People of Healthcare

A MEDINN INITIATIVE · KERALA

Interview guide and leverage playbook

For my own use. How to run each of the 50 conversations, what to ask, and how to turn each one into introductions, problem statements, mentors, pilots and, over time, new startups in Kerala.

1. The aim, in one line

> Understand the gaps in Kerala's healthcare ecosystem properly, publish what I learn (with consent), and use each relationship to connect clinical problems with the people who can solve them.

The progression: **Observation → Knowledge → Awareness → Connection → Collaboration → Opportunity.** Each conversation moves someone one step along it.

2. Before each call (15 minutes)

- Read what they've built: website, one or two news items, their LinkedIn. Note **one specific thing** to mention.
- Choose their segment (hospital, device maker, diagnostics, biotech, digital health, investor, KOL) and pick 6 to 8 questions below, not all of them.
- Send the brief (PDF) the day before. Confirm the time and how they prefer to meet (call, video, in person).
- Have a note ready for where to log it (Notion: People of Healthcare: Nominations).

3. Shape of the conversation (30 to 45 minutes)

Time	What
5 min	Thank them. Who I am, in 30 seconds. Why People of Healthcare. Ask permission to take notes.
10 min	Their story: how they started and what they're building now.
15 min	The five lenses: choose the ones that fit them.
5 min	Doctors and students: how clinicians can take part.
5 min	Close: who to meet next, one problem they'd love solved, and agreeing next steps.

4. Question bank

Opening: their story

- How did you get into healthcare, and what are you building or leading now?
- What problem were you trying to solve when you started? What did the first year look like?
- What are you proudest of, and what has been the hardest part?

People

- Who has shaped your work in Kerala? Who is quietly building something important that I should meet?
- Who are the three people I must speak to next, and would you introduce me?

Production: what Kerala can make

- What does Kerala do well today in your area? What do you still depend on other states or countries for?
- What would it take to make [a product you use] in Kerala? What are the real barriers: cost, approvals, supply, skills?
- Where do you see the biggest opportunity for local manufacturing or production?

Talent

- Which roles are hardest to fill: biomedical engineers, service technicians, regulatory, quality, product, data?
- What do fresh graduates, doctors or engineers lack when they join you? How do you train and keep people?
- Which institutions send you the best people? What would you like colleges to teach?

Innovation: from problem to product

- Tell me about something that began as a clinical observation and became a product or service. What happened at each step?
- Where do ideas usually stall: prototype, validation, approval, first customer, funding?
- How do you work with clinicians today? What would make that easier?

Pipelines: funding, mentors, partners, approvals

- What helped you most: funding, mentorship, a partner, a hospital? What was missing?
- Which approvals or regulatory steps were hardest to navigate? What do you wish you had known earlier?
- What does a hospital need to see before it adopts a new product?

The 2026 themes: AI, devices, collaboration

- Where is AI genuinely useful in your work today, and where is it hype?
- How is the focus on medical devices changing your priorities?
- How is medicine and engineering collaboration working in practice? What is the missing link?

Business and the bigger picture

- Which business models work in Kerala healthcare, and who ends up paying?
- If you could fix one thing in Kerala's healthcare ecosystem, what would it be? What problem is nobody working on?
- What would make Kerala a better place to build healthcare companies?

Doctors and medical students

- How can clinicians contribute meaningfully: as advisors, collaborators, product developers, researchers, entrepreneurs?
- What should a medical student or house surgeon know before approaching you?

Closing

- What should I have asked you?
- Is there anything off the record, or anything you'd rather I don't publish?

5. Working with their stature: the ask ladder

Principle: **give first, ask small, one ask at a time, and never press.** Move up the ladder only as trust builds. People are most generous with introductions and problems, so start there.

Level	The ask	How to say it
0. In the call	3 introductions. 1 to 3 real unsolved problems. What they would buy or adopt.	*"Who else should I meet?" · "What is one problem you wish someone would solve for you?"*
1. After the profile	Approval and a quote. Share the profile on their own channels. A short talk or Q&A for medical students.	*"Would you be open to sharing this with your network, and to a 20-minute session with medical students at GMC Kozhikode?"*
2. Mentoring and access	A mentor slot for student or founder teams. Internships or projects for students and engineers. A clinical or technical advisor role for IEDC GMC Kozhikode or MedTech Malabar.	*"Could one of your team mentor a student project for a month?"*
3. Pilots and validation	Hospital founders: pilot sites and clinical access. Manufacturers: clinician feedback and co-testing. Both: a named contact to work with.	*"If a team brings a tested prototype for a problem like this, would you host a small, supervised pilot?"*
4. Convening	A closed round-table of 8 to 10 of the 50. A shared problem bank. Co-hosting a hackathon or challenge. Funding or prize support. Introductions to angels, grants and CSR.	*"I'm bringing a few of the people I've met into one room. Would you join?"*
5. Building startups	A cohort run through a clear path: clinical problem, team, prototype, validation at a host hospital, incubation, first customer, investor introductions.	*"Would you back a team from this pipeline as mentor, pilot site or first customer?"*

6. From conversations to startups: the funnel

Stage	What it looks like	What I keep
1. Conversation	The interview. Trust begins.	Notes, key insight, segment.
2. Profile	A published story, approved by them.	The profile and its links.

3. Problem bank	Real unsolved problems collected from all 50.	A list of problems, with who raised them.
4. Matchmaking	Match problem, team, mentor and pilot site.	Who offered what.
5. Pilot	A supervised, small trial at a hospital or with a manufacturer.	Outcome and learning.
6. Startup	A team continues with incubation and funding introductions.	Names of teams and supporters.

Add these fields to the Notion database: **Date met** · **Key insight** · **Problems shared** · **Introductions offered** · **Offers (mentor / pilot / internship / funding intro)** · **Profile status** · **Follow-up date**. Count what is real; never publish a number that is not.

7. After each call

- **Within 24 hours:** a thank-you message with one thing you learned and one thing you will do.
- **Within 48 hours:** log it in Notion. Draft the profile.
- **Within a week:** send the profile for approval. Publish once approved. Tag and thank them.
- **After 30 days:** a short update on what happened with their introductions or problems.

> *"Thank you for your time today. What stayed with me was [one specific thing]. I'll send you a draft of your profile within a week for your approval. And I'll reach out to [name] as you suggested."*

8. What the insights become

- A profile for each person (People of Healthcare, the Medinn blog, LinkedIn and Instagram).
- Articles on the gaps, in the same spirit as the series: for example who makes medical devices in Kerala, the talent gap in MedTech, and the road from a clinical problem to a product here.
- A shared problem bank that student teams, hackathons and MedTech Malabar can work from.
- A year-end summary of what I learned (a proposal, to be written once the conversations are done).

9. Ground rules for trust

- Consent first: nothing is published without their approval. Respect anything off the record.
- Be transparent that Medinn is a consulting practice. No payment to be featured, and no promises of business, funding or outcomes.
- Be accurate and balanced: feature what they have built, not a sales pitch, and no invented numbers or quotes.
- Respect their time: be on time, finish on time, and ask for one thing at a time.
- Give back: share their work, make introductions in return, and report back what happens.

Dr. Sidharth Narayanan · House Surgeon, GMC Kozhikode · Founder, Medinn · medinn.in/people-of-healthcare · +91 94005 13128 · medinnnetworking@gmail.com